

HOW TO OPEN A GROUP HOME BUSINESS in Tennessee

Family model & Community Living Supports under ECF CHOICES — serving adults with developmental disabilities.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The Tennessee model — read this first

“Group home” in Tennessee means providing residential support to adults with intellectual and developmental disabilities, and the state funds it through Employment and Community First (ECF) CHOICES, a program run by TennCare (the state Medicaid agency) and delivered through private managed care organizations (MCOs). Two things make Tennessee distinctive, and they run through everything below. First, **MANAGED CARE**: every ECF CHOICES member is enrolled with an MCO, so to get paid you get **CREDENTIALLED** by DIDD and then **CONTRACT** with the MCOs — there is no billing the state directly. Second, the main residential services are **FAMILY MODEL RESIDENTIAL** (an adult lives in a private home with a trained caregiver) and **COMMUNITY LIVING SUPPORTS / SUPPORTED LIVING** (a home supporting up to four adults with staff). Also important: Tennessee's older 1915(c) waivers are closed to new enrollment — all new members come through ECF CHOICES.

***The two ways in.** Path A — provide family model residential (host home): one adult (sometimes two) lives in a private home, you complete a DIDD home study, and you usually work under a credentialed agency. Smallest, fastest, closest to a host home. Path B — become your OWN credentialed provider agency: get credentialed by DIDD, contract with the MCOs, and run Community Living Supports / supported living homes (up to four residents) with DIDD life-safety inspections. A much bigger lift — but it's the real business. This guide walks both, and is honest about which is realistic when.*

***The names you'll deal with.** DIDD = the Department of Intellectual and Developmental Disabilities (now within the Department of Disability and Aging) — it credentials providers and inspects homes. TennCare = Tennessee's Medicaid agency; it runs ECF CHOICES and sets the rates. MCOs = the managed care organizations that administer ECF CHOICES and PAY you — BlueCare, Amerigroup/Wellpoint, and UnitedHealthcare. Support Coordinator = the MCO staffer who develops the person-centered support plan. DDA Regional Intake Office = the front door for eligibility and self-referral. ECF CHOICES = Employment and Community First CHOICES, the managed-care program that funds all of this.*

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened a home — every time we name a thing, we stop, break it down, and tell you exactly where to go and what to ask. It is general information, not legal, tax, or billing advice; requirements, rates, and codes change, so verify each step with DIDD, TennCare, the MCOs, your local fire authority, and qualified local counsel (see Part 12) before you rely on it.

The journey at a glance

These milestones take you from idea to your first payment — whether you start with a family model home or build your own agency.

1	Decide your path & set up the business	Path A (family model) or Path B (credentialed agency); entity, EIN, NPI, insurance (Parts 3–4)
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2	Learn the ECF CHOICES standards	Read the DIDD Credentialing Protocol and the MCO provider manuals (Part 5)
3	Get credentialed by DIDD	Application + Settings Rule self-assessment; the committee meets monthly (Part 5)
4	Contract with the MCOs	BlueCare, Amerigroup/Wellpoint, UnitedHealthcare — they pay the claims (Part 5)
5	License & inspect the home	Setting-size limits, HCBS rights, DIDD life-safety inspection (Part 6)
6	Staff & write your policies	Direct support professionals, RN oversight, documentation (Parts 7–8)
7	Get a person placed	ECF CHOICES enrollment, assessment, MCO match & support plan (Part 9)
8	Deliver services & get paid	Bill the MCO against the authorized support plan, within 120 days (Part 10)

Rule of thumb. *Start smaller than you think. Family model residential — one adult living in a private home — is the fastest way in and the easiest to learn on, often under an established agency that holds the ECF CHOICES contracts and handles the billing. Many providers begin this way, learn the managed-care system, and grow into their own DIDD credentialing and their own supported living homes.*

Before you begin — a realistic picture

Two honest truths will save you time and money. First, this is a relationships-and-documentation business funded by Medicaid managed care, not a real-estate play or a normal small business: your revenue depends on getting credentialed by DIDD and contracted with the MCOs, and then on ECF CHOICES members choosing you — so your reputation with DIDD, established providers, and MCO Support Coordinators is your most valuable asset. Second, the money arrives on a chain and on a delay: DIDD credentialing runs through a committee that meets monthly, MCO contracting takes more time on top of that, and you don't bill until a person is placed and their services are authorized — so you need patience and a working-capital cushion to carry any staff and setup costs through the ramp. Start on Path A under an established provider if you can, learn the system on someone else's credentialing, and grow into your own agency once you know the terrain.

Here's how the rest of this guide is organized: Parts 1–2 explain what the business is and who regulates and pays you; Part 3 sets up the business itself, with the EIN, NPI, bank account, and insurance broken all the way down; Part 4 walks the two paths in; Part 5 is the credentialing-and-contracting chain; Part 6 covers the home and fire safety; Parts 7–8 are staffing and the policies and records DIDD reviews; Part 9 is how a person actually gets placed with you; Part 10 is how you get paid; Part 11 covers costs, timeline, and getting placements; and Part 12 collects every phone number and script in one place. Read it once end to end, then keep it beside you as a checklist.

PART 1

What This Business Is

In short: *Providing residential support to adults with developmental disabilities under ECF CHOICES — either family model (one adult in a private home) or Community Living Supports / supported living (up to four adults with staff). Paid by Medicaid through the MCOs, after DIDD credentials you.*

A “group home” business in Tennessee is really a developmental-disabilities support business. You help an adult with an intellectual or developmental disability live successfully in the community — with daily support, supervision, skill-building, help with health and medications, and a real home life — rather than in an institution. Tennessee’s main residential ECF CHOICES services for this span a spectrum: from Community Living Supports – Family Model (CLS-FM), where an adult lives in a private home with a trained caregiver or family, to Community Living Supports (CLS) / supported living, where a home supports up to four adults with staff — 24/7 awake staff for supported living.

Be clear-eyed about what kind of business this is. This is a human-services business built on relationships and documentation, funded by Medicaid through managed care — not a real-estate play. Your value is the quality and reliability of the support you provide and your ability to keep it compliant and well-documented, because that’s what DIDD credentials, what the MCO authorizes and pays for, and what keeps people safe. Go in understanding that the care and the paperwork ARE the business.

Why this business — and why now

Demand for community-based services for people with developmental disabilities is high and durable: Tennessee, like every state, is moving people out of institutions and into the community, families overwhelmingly want a real home for their loved one, and ECF CHOICES funds exactly this. Because every member is enrolled with an MCO and members choose their provider, a reliable, well-run provider that builds relationships with Support Coordinators can keep its homes full. And you can start lean — providing family model residential for one person under an established agency — and grow into your own credentialed agency as you learn the system.

Be honest about the two realities that trip up new providers. (1) *Managed care adds steps: you don’t just get a state license and bill Medicaid — you get credentialed by DIDD AND contract with each MCO before a dollar flows.* (2) *Room and board is separate from your pay: ECF CHOICES pays for the SERVICE, not the person’s rent and food, which come from their own income (usually SSI/SSDI). Plan your business — and explain it to families — around both.*

PART 2

Who Regulates You & Who Pays You

In short: *Two bodies stand between you and a paid claim: DIDD credentials you as a provider, and the MCO authorizes and pays for services under ECF CHOICES, which TennCare administers. The MCO's Support Coordinator builds each person's plan.*

DIDD — Department of Intellectual and Developmental Disabilities (your credentialer)

DIDD (now within the Department of Disability and Aging) is the credentialing authority for ECF CHOICES providers — a role it has held since 2023. It sets the credentialing protocol, reviews applications through a Provider Credentialing Committee that meets monthly, and inspects homes through its Housing Inspectors. Without DIDD credentialing you cannot serve ECF CHOICES members. Start at tn.gov/disability-and-aging under “Become a Credentialed Provider.”

TennCare — Tennessee's Medicaid agency (the program & the rates)

TennCare administers ECF CHOICES — it defines the services, sets the rates, and contracts with the MCOs that deliver the program. You won't bill TennCare directly, but its ECF CHOICES service definitions and rate structure govern everything the MCOs authorize and pay. The program authority sits in TennCare rule 1200-13-01 and T.C.A. § 33.

The MCOs — managed care (your payer)

Because every ECF CHOICES member is enrolled with an MCO, you contract with the MCOs whose members you want to serve: BlueCare, Amerigroup/Wellpoint, and UnitedHealthcare. Each has its own contracting and billing system. The MCO authorizes services in the person's support plan and pays your claims, and the MCO's Support Coordinator develops the person-centered support plan and, with the person's choice, matches them to your home.

What this means for you: the path to payment is a chain — DIDD (credential) → MCO (contract, authorize, pay) — and each link takes time. Get credentialed first, because an MCO won't contract with you until you are, then contract with as many of the three MCOs as you want to serve. Treat the MCO Support Coordinators as key relationships, because their members choose their providers. A home that's credentialed, contracted, and known to Support Coordinators is a home that fills.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In Tennessee you form the LLC through the Secretary of State. Your entity, EIN, and NPI all have to be in place and consistent before DIDD credentialing and MCO contracting, because they're checked along the way. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Articles of Organization with the Tennessee Secretary of State.** Online at sos.tn.gov — the LLC filing fee is \$50 per member, with a \$300 minimum and a \$3,000 maximum. Name the LLC and list a registered agent (this can be you).
- 2. Adopt an Operating Agreement.** The internal document that sets ownership and management — your bank, your DIDD credentialing file, and your MCO contracts will reference your structure.
- 3. File the annual report every year.** Tennessee LLCs file an annual report with the Secretary of State — \$50 per member, \$300 minimum — due by the first day of the fourth month after your fiscal year-end, to stay in good standing.
- 4. Confirm local business licenses.** Tennessee has a state/county business tax; check what city or county business licenses apply where you'll operate.

No newspaper publication in Tennessee. *Tennessee has no LLC publication requirement — forming the entity is a straightforward Secretary of State filing plus the annual report. Budget for the \$300 minimum both to form and to renew each year.*

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to identify your agency to DIDD and to bill the MCOs for ECF CHOICES services. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.

2. Start a Type 2 (Organizational) application. Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the taxonomy that fits a developmental-disabilities / community-based residential provider — confirm the exact one DIDD and the MCOs want. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of Tennessee branches — First Horizon, Regions Bank, Pinnacle Financial Partners, Truist, FirstBank, or a local credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

A residential DD provider brings staff into people's homes and supports vulnerable adults, so you need coverage built for a human-services provider — and DIDD and the MCOs will expect proof:

Coverage	What it protects against	Must-have?
General & Professional Liability	Injury or property damage, plus claims about the SUPPORT and supervision you provide	Yes — foundational
Abuse & Molestation	Allegations of abuse of a person you support — general liability EXCLUDES this	Yes — high exposure
Directors & Officers / Management	Regulatory, employment, and governance claims	Recommended for an agency
Non-Owned & Hired Auto	DSPs driving their own cars to support or transport the people you serve	Yes — critical
Workers' Compensation	DSP injuries	Required in TN (5+ employees)
Property	Coverage on any provider-owned or leased home	If you own/lease a home

Abuse & molestation is your highest exposure. Supporting vulnerable adults means abuse & molestation coverage — EXCLUDED by standard general liability — is essential, with a real limit, not a token sub-limit. Confirm it explicitly.

Real places to get it

Buy through an independent agent or broker who writes HUMAN-SERVICES / developmental-disabilities provider programs — this is a specialty niche. Carriers and programs active in DD/IDD services include NSM/Irwin Siegel Agency (a social-services specialist), Negley Associates, Philadelphia Insurance (PHLY), and CNA. Find a local Tennessee agent at [trustedchoice.com](https://www.trustedchoice.com) and ask specifically for a developmental-disabilities / ECF CHOICES provider program. Confirm the carrier writes this in Tennessee before you rely on a quote.

What to ask the insurance agent

“Do you write developmental-disabilities / supported-living providers in Tennessee — which carriers, and what's the minimum premium?”

“Is abuse & molestation included, at what limit, and is it a sub-limit or a full limit?”

“Do you include non-owned & hired auto for my DSPs driving the people we support?”

“What limits do DIDD and the MCOs expect on my contracts?”

“Can you set up Tennessee workers' comp for my direct support professionals?”

3.6 Keep your paperwork consistent — every step downstream checks it

One practical warning that saves Tennessee providers real time: your legal name, ownership, EIN, and NPI have to line up EXACTLY across every step that follows — your DIDD credentialing application, your Settings Rule self-assessment, and each of your MCO contracts. Managed care means your information passes through several organizations before a claim pays, and a mismatch at any handoff — a slightly different business name here, an old address there — is one of the most common and most frustrating causes of a stalled credentialing file or a rejected claim. Set the entity up once, correctly, and make every downstream document point at the same clean legal picture. It's worth an hour with your accountant or attorney up front to confirm the structure, because untangling a mismatch after you've applied costs far more than getting it consistent now. And keep your DIDD credentialing and MCO contracts calendared for renewal — letting any one of them lapse stops your payments even when the care is going perfectly.

The business foundation to have in hand before you apply for credentialing:

- Your formed Tennessee LLC (Articles of Organization filed, annual report current) and Operating Agreement
- Your EIN confirmation (CP-575) from the IRS
- Your Type 2 (organizational) NPI from NPPES
- A business bank account opened in the entity's exact legal name
- Bound insurance — general/professional liability, abuse & molestation, non-owned auto, and workers' comp
- A single, consistent legal name, ownership list, and address used identically on every document

With those six things in place and consistent, you're ready to build the credentialing application in Part 5 on a clean foundation — and you've removed the paperwork mismatches that stall more Tennessee applications than any substantive problem does.

PART 4

The Two Paths — Family Model (A) or Credentialed Agency (B)

In short: Path A is the fast, small way in — provide family model residential (host home), often under an established credentialed agency. Path B is the real business — become your own DIDD-credentialed agency running supported living homes. Pick the one that fits where you are today; many start with A and grow into B.

4.1 Path A — family model residential (host home)

Family model residential — billed as Community Living Supports – Family Model (CLS-FM) — means an adult with a developmental disability lives in a private home with a trained caregiver or family and shares everyday life, rather than living in a facility. It's the fastest way in and the closest thing to a host home. Everyone in the household is part of it, so it's a family decision.

- 1. Make sure it fits your life.** Family model means an adult lives with you and shares your everyday life. Everyone in the household is part of the process — go in with eyes open about the good days and the hard ones.
- 2. Connect with a credentialed provider agency.** Most family model caregivers work with an established, DIDD-credentialed agency that holds the ECF CHOICES contracts, supports you, and handles the billing. Ask your DDA Regional Intake Office for agencies in your area, and interview more than one — how they support and pay family model caregivers varies.
- 3. Complete your DIDD home study.** Family model (CLS-FM) providers must complete a DIDD home study before anyone is placed — it looks at your home, your household, and the match. This is the heart of family model approval.
- 4. Pass background checks & training, then get matched.** Complete the criminal background check, the DIDD training, CPR/First Aid, and abuse-prevention training; then the agency and the person's Support Coordinator match you with someone whose needs and personality fit your home, built around person-centered planning and the person's choice.

The difficulty-of-care tax note. Because the person you support lives in your OWN home, your family model payments may qualify for the IRS “difficulty-of-care” exclusion (potentially non-taxable). Confirm with a tax professional — it can meaningfully change the economics of a family model arrangement. Many family model caregivers also serve as the member's representative payee for their room-and-board money.

4.2 Path B — become your own DIDD-credentialed agency

Path B is more work, more money, and more control: you get credentialed by DIDD, contract with the MCOs, and run Community Living Supports / supported living homes (up to four residents) — with DIDD life-safety inspections of each home. It's the real, scalable business — and Part 5 walks the credentialing path step by step. Most operators who go this route either have direct experience in DD services or hire someone who does, because your application has to show DIDD you understand the standards and can deliver — including, for higher-need homes, the master's-level clinician and psychiatry access the standards require.

PART 5

Get Credentialed by DIDD & Contracted with the MCOs

In short: For Path B, this is the chain that unlocks payment: learn the standards, apply for DIDD credentialing (the committee meets monthly), then contract with the MCOs whose members you'll serve. Credentialing is the gate you must pass before any MCO will contract with you.

- 1. Learn the standards cold.** Read the DIDD Credentialing Protocol, the ECF CHOICES service definitions from TennCare, and the MCO provider manuals (BlueCare, Amerigroup/Wellpoint, UnitedHealthcare) closely — your application and your policies must reflect them and describe HOW you'll deliver services.
- 2. Apply for DIDD credentialing.** Submit your credentialing application and the required HCBS Settings Rule self-assessment. The Provider Credentialing Committee reviews applications and meets monthly, so plan around that cadence. This is the gate you must pass before an MCO will contract with you.
- 3. Contract with the MCOs.** Credentialing lets you serve, but the MCOs pay you — so contract with one or more of the three ECF CHOICES MCOs (BlueCare, Amerigroup/Wellpoint, UnitedHealthcare). Each has its own contracting and billing system; contract with as many as you want to serve their members.
- 4. License & inspect your homes.** For supported living or residential habilitation, hold the appropriate DIDD license, and DIDD Housing Inspectors complete a life-safety inspection of each new home before occupancy (and again every 24–30 months). See Part 6.

What this means for you: credentialing is a quality gate, not a form. DIDD wants evidence — real policies, real service descriptions, real capacity — that you can safely support people, and it checks that your homes meet the federal HCBS Settings Rule (this is someone's home, with lease-like protections). The providers who get through cleanly treat the credentialing protocol and the MCO manuals as the blueprint for the whole operation, so building your policies and staffing to the standard does double duty: it earns your credentialing AND it's how you'll actually run.

PART 6

The Home — Settings, HCBS Rights & Fire Safety

In short: A family model home is approved through a DIDD home study — a private residence, not a licensed facility. A supported living home (up to four adults) is licensed and passes a DIDD Housing life-safety inspection. In every case, it must genuinely be the person's home, with lease-like protections.

6.1 How Tennessee handles the home

A family model home is approved through a DIDD home study — it's a private residence, not a licensed facility. A supported living or residential habilitation home is inspected by DIDD Housing Inspectors, who complete a life-safety inspection before anyone moves in and then re-inspect every 24–30 months. Those inspections check fire and emergency safety AND HCBS-settings compliance — because this is someone's home, with lease-like protections: a residency agreement, privacy, choice, and full access to the community.

6.2 “Do I need fire sprinklers?” — the honest answer

Family model homes (one, sometimes two people) and CLS/supported living homes (up to four) are small residences, so commercial fire-sprinkler systems generally are NOT required. What you must pass is the DIDD life-safety inspection, which looks at fire and emergency safety — exits, alarms, and evacuation — for the home. Actual sprinkler and building requirements come from your local fire authority and building code and depend on the home's size and occupancy, so confirm anything unusual (a larger or modified home) with DIDD Housing and your local fire authority BEFORE you commit to a property.

6.3 The home checklist

- ☐ For family model: a completed DIDD home study of your home and household (and re-checks as required).
- ☐ For supported living / residential habilitation: a DIDD Housing life-safety inspection before occupancy, then every 24–30 months.
- ☐ HCBS settings rights: a lease or residency agreement, privacy, choice, and full community access — keep a copy of each member's agreement for audits.
- ☐ Working smoke and carbon-monoxide detectors, a fire extinguisher, and clear exits with a practiced evacuation plan.
- ☐ A safe, clean, sanitary home — sound utilities and electrical, working plumbing, and safe food storage and preparation.
- ☐ Setting-size limits respected — no more than four service recipients in a CLS/residential-habilitation home (three in some blended-home situations), regardless of program or funding source.

Two things people get wrong. (1) Home capacity is capped — no more than four service recipients may live in a CLS/residential-habilitation home, regardless of program or funding source, so plan your capacity accordingly. (2) Room and board is separate from your pay — the service rate doesn't cover the person's rent and food (those come from their own income, usually SSI/SSDI). Keep the two completely separate in your books and your agreements; many family model caregivers also serve as the member's representative payee.

PART 7

Who You Actually Need — Staffing

In short: *In family model (Path A), you and your household provide the support, with a credentialed agency's oversight, home study, and billing. As a credentialed agency (Path B), plan for a director, direct support professionals, clinical and nurse oversight, and billing — one person can wear several hats at the start.*

7.1 The roles

- **Agency director / administrator:** runs the agency and owns DIDD credentialing and the MCO contracts.
- **Direct support professionals (DSPs):** the caregivers in your homes — supported living requires awake staff 24/7.
- **Master's-level clinician & psychiatry access:** required for homes serving members with significant behavioral or mental-health needs.
- **Registered Nurse:** for health oversight and medication — often part-time or contracted at first.
- **Billing / administrative support:** handles MCO billing (EDI claims within 120 days) and documentation.

7.2 Recruiting and keeping DSPs — the real make-or-break

Direct support professionals are the heart of this business and the hardest thing to keep. DSP turnover is high across the whole field, and a supported living home that can't reliably staff its awake shifts can't safely support people or keep its authorizations — so treat recruiting and retention as your core operation, not an afterthought.

- **Recruit continuously and hire for heart:** specific tasks can be trained; patience, reliability, and genuine care for people with disabilities cannot. Check references for dependability.
- **Onboard to the standard:** real orientation, the required background check and training (including CPR/First Aid and abuse-prevention), and a competency check before a DSP supports someone alone — this protects the people you serve and satisfies DIDD.
- **Retain deliberately:** consistent schedules, correct and on-time pay, real support when a DSP hits a hard day, and recognition. Every DSP who quits costs you re-hiring, re-training, and often continuity for the person they supported.

PART 8

Policies, Records & Staying Compliant

In short: *Your policies and documentation are what DIDD reviews and what the MCO pays against. Build them to the ECF CHOICES standard from day one — medications, emergencies, incident reporting, and the rights of the people you support (including their lease/residency protections).*

You keep clear documentation covering medications, emergencies, incident reporting, and the rights of the person you support — including their lease/residency protections under the HCBS Settings Rule — and the person-centered support plan drives what you deliver and record. This documentation is both the DIDD credentialing requirement and the backbone of a safe, well-run operation. Build a simple system that keeps each person's plan, service records, and incident documentation current, because DIDD reviews it and the MCO can audit it.

8.1 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 9

The Person's Journey — Referral to Move-In

In short: *You can't get paid until an MCO member is placed with you and services are authorized. The path runs from eligibility, through ECF CHOICES enrollment and assessment, to the MCO Support Coordinator's person-centered support plan and the match to your home.*

- 1. The person qualifies.** They have an intellectual disability (before age 18, IQ under 70) or a related developmental disability (before age 22), are a Tennessee resident, and have (or are eligible for) TennCare. Eligibility and "benefit group" are set by assessment — the person doesn't pick their group. (DDA Regional Intake helps with self-referral.)
- 2. They enroll in ECF CHOICES.** All new enrollment is through ECF CHOICES — the old 1915(c) waivers are closed to new members. Funding is limited, so not everyone who applies can enroll right away.
- 3. The plan is built.** The MCO's Support Coordinator develops the person-centered support plan, which identifies residential supports (family model or CLS/supported living) at the appropriate assessed level.
- 4. The match and authorization.** You (or your agency) are matched to the member based on fit and your credentialed services, with the person's choice; the MCO authorizes the service and the level.
- 5. Move-in.** With the plan and authorization in place, the person moves in and you're delivering a billable service.

What this means for you: because members choose their provider and funding is limited, your census depends on relationships and reputation, not marketing. Get known to DIDD, to established providers, and above all to MCO Support Coordinators as the provider who supports people well, communicates, and handles a placement smoothly — and the matches follow.

Plan your business around the enrollment reality. *ECF CHOICES funding is limited and members enroll on their own schedule — which shapes how you should plan. Don't build a business model that assumes you'll fill four beds the month you're credentialed; assume a gradual build as Support Coordinators learn to trust you and matches come through. Two practical moves help: start on Path A providing family model residential under an established agency so you have revenue while you build, and stay in regular, helpful contact with the Support Coordinators in your area so that when one of their members is choosing a provider, you're the name they think of first. Patience plus reliability is the whole strategy — the demand is real, but it arrives one authorized person at a time.*

PART 10

How You Get Paid

In short: Two keys unlock payment — DIDD credentialing and the MCO contract — and the MCO pays an ECF CHOICES rate set by TennCare by the member's assessed level. Room and board is separate, and you bill only for days the member actually received the service.

10.1 The keys and the rate

- **DIDD credentialing:** the state's quality gate — without it you can't serve ECF CHOICES members, and no MCO will contract with you.
- **The MCO contract:** the MCO actually authorizes and pays for services — you contract with one or more of BlueCare, Amerigroup/Wellpoint, and UnitedHealthcare, and bill each for its own members.
- **The rate:** ECF CHOICES rates for residential supports are set by TennCare by the member's assessed level (the ECF CLS levels), based on medical-necessity criteria — not by how many people live in the home. Family model (CLS-FM) and CLS/supported living have their own rate structures. The rate is inclusive of most transportation. Exact codes and current rates are in each MCO's provider manual and TennCare's ECF CHOICES materials — confirm the current figure before you budget.

10.2 The money math — how residential supports pencil out

ECF CHOICES economics are different from an hourly private-pay business — your revenue is the authorized rate for the assessed level, and your biggest cost is DSP labor. Lay it out with your own numbers:

- **Revenue:** the residential support authorized in each member's support plan, billed to their MCO at the TennCare rate for the assessed level — paid only for days the member actually receives the service (not days in a hospital or on therapeutic leave).
- **Cost:** DSP wages and payroll taxes are the bulk — heavier for supported living because it requires awake staff 24/7 — then RN and clinical oversight, administration, insurance, and (for a home you own or lease) the building.
- **The lever:** because the rate is set by the member's level, your margin comes from staffing efficiently to each person's authorized plan and keeping your homes filled with authorized people — utilization, not rate, is the number you manage.
- **Room and board stays separate:** the person's rent and food come from their own income (usually SSI/SSDI), NOT ECF CHOICES — keep that money and those agreements entirely separate from your service billing.

10.3 Your first payment

1. **Confirm the service and level are authorized in the person's support plan and by their MCO.**
2. **Deliver the service and keep your daily documentation.**
3. **Submit the claim to the MCO electronically (EDI) — within 120 days of the date of service.** Your agency does this if you're working under one; otherwise you bill per your MCO contract.
4. **Get paid — and bill only for days the member actually received the service.** Keep your credentialing, MCO contracts, and authorizations current so payment never lapses.

PART 11

What It Costs, How Long It Takes & Getting Placements

In short: *Path A can start relatively quickly; Path B takes several months through credentialing and contracting. Placements come through the MCOs, so your relationships with Support Coordinators are your census.*

11.1 Timeline & startup costs

Family model residential (Path A) can move relatively quickly once you've chosen an agency — the home study, background checks, and training. Becoming your own credentialed agency (Path B) takes longer: the business setup, DIDD credentialing (the committee meets monthly), contracting with each MCO, and licensing and the life-safety inspection of each home — plan for several months end to end. Family model costs are modest (mainly your time, background checks, training, and getting your home ready); a credentialed agency adds business formation (\$300+ to form, \$300+ each year), insurance, the clinical supports supported living requires, staff payroll (24/7 awake staff), licensing and inspection, and your policies and procedures. Confirm current fees with DIDD and the MCOs.

11.2 Getting your first placements

Placements don't come from advertising — they come through the MCO process and your reputation. Here's a simple plan:

- 1. Get known to DIDD and established providers.** Call your DDA Regional Intake Office and — if you're starting on Path A — connect with established credentialed agencies that bring on family model caregivers and supported living staff.
- 2. Build relationships with MCO Support Coordinators.** They develop each member's support plan and match members to providers, and members choose. Be the provider they trust to handle a placement well, communicate, and never leave a shift unstaffed.
- 3. Be responsive and reliable.** Answer the phone, say yes to a good match, handle move-in smoothly, and follow through — one good placement earns the next referral.

PART 12

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: DIDD and the MCOs are the center of gravity, with your DDA Regional Intake Office and local fire authority close behind. Statewide contacts are the same for everyone; your fire/building offices are local. Here's the list and the scripts.

12.1 Statewide (same for everyone)

For...	Contact
Becoming a credentialed provider	DIDD (Dept. of Disability & Aging) — tn.gov/disability-and-aging → Become a Credentialed Provider
Getting paid (contract & billing)	The MCOs — BlueCare, Amerigroup/Wellpoint, UnitedHealthcare (800-690-1606)
Which MCO / member help	TennCare MCO line — 1-855-259-0701
Eligibility & self-referral help	DDA Regional Intake Office — West (866) 372-5709 · Middle (800) 654-4839 · East (888) 531-9876
Program rules & rates	TennCare ECF CHOICES materials · rule 1200-13-01 · T.C.A. § 33
Business, EIN & NPI	sos.tn.gov · irs.gov (EIN) · nppes.cms.hhs.gov (NPI)

12.2 The local calls — and exactly what to ask

① DIDD credentialing / a credentialed agency — your most important calls

"I want to become a family model provider / a credentialed CLS agency. What's the credentialing process, and what's the current provider need in this area?"

"(To an agency) How do you support family model caregivers, how do you match, and how and when do you pay?"

② DIDD Housing — the life-safety inspection

"What does the life-safety inspection check for a supported living home, and can I get the inspection tool to self-assess before I open?"

③ Local fire authority / building — the home

"I'll operate a small residential home for up to four adults with disabilities. What fire-safety requirements apply, and is anything needed beyond alarms, extinguishers, and exits?"

12.3 Worked examples — Tennessee's two biggest metros

DIDD credentialing and the MCO process are statewide — the local piece is mainly your fire authority and your DDA region. Tennessee is divided into three DDA regions — West, Middle, and East — each with an intake office.

- **Nashville (Middle Tennessee):** DDA Regional Intake — Middle Tennessee (800) 654-4839; DIDD statewide for credentialing, then contract with the MCOs; Metro Nashville/Davidson County fire authority for fire-safety requirements and DIDD Housing for the life-safety inspection. The quirk: Nashville is the state's busiest market — build relationships with MCO Support Coordinators, since members choose their provider.
- **Memphis (West Tennessee):** DDA Regional Intake — West Tennessee (866) 372-5709; same statewide DIDD credentialing, then MCO contracts; City of Memphis / Shelby County fire authority, and DIDD Housing for the life-safety inspection. The quirk: confirm whether your address is inside the City of Memphis or unincorporated Shelby County — it changes your fire and building contacts.

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I have to become a credentialed agency, or can I start smaller?

You can start smaller — Path A lets you provide family model residential (host home), usually as a caregiver under an established DIDD-credentialed agency that holds the ECF CHOICES contracts and handles the billing. It's the fastest way in and the best way to learn the managed-care system before you take on your own DIDD credentialing (Path B). Many providers grow from A to B.

Why do I have to deal with an MCO — isn't this just Medicaid?

Tennessee runs its I/DD services through managed care, so every ECF CHOICES member is enrolled with an MCO (BlueCare, Amerigroup/Wellpoint, or UnitedHealthcare). The MCO coordinates the person's care through a Support Coordinator and actually authorizes and pays for services. So the payment chain is: DIDD credentials you, then you contract with the MCOs and bill them — there's no billing the state directly.

Does ECF CHOICES pay for the person's rent and food?

No — the service rate pays for the SUPPORT you provide, not room and board. The person's rent and food come from their own income, usually SSI/SSDI. Keep the two completely separate in your agreements and your books; many family model caregivers also serve as the member's representative payee. Mixing them is a common and serious mistake.

How big can a group home be?

Capped at four. No more than four service recipients may live in a CLS/residential-habilitation home, regardless of program or funding source (three in some blended-home situations). Tennessee's model is small, community-based homes — plan your capacity around that, and grow by adding homes, not by making one home larger.

You can do this

Opening a group home business in Tennessee is real work — the managed-care system adds a couple of extra steps — but it's a real, walkable path, and people across the state do it every day, giving someone a real home in the community. You don't have to be a lawyer or hire a consultant. Whether you start with a family model home or build your own credentialed agency, take it one step at a time: set up the business, learn the ECF CHOICES standards, get credentialed by DIDD, contract with the MCOs, ready the home, and build the relationships that bring you placements. Verify the current details with the State and the MCOs as you go, and lean on your DDA Regional Intake Office — that's what they're there for.

Reminder. Prepared for PolicyReady.org and grounded in Tennessee DIDD and TennCare rules, the ECF CHOICES program materials, the DIDD Credentialing Protocol, and the MCO provider manuals (BlueCare, Amerigroup/Wellpoint, UnitedHealthcare) as of 2026. This is general information, not legal, tax, or billing advice. Requirements, rates, and codes change — always verify current details with DIDD, TennCare, the MCOs, and qualified counsel before you act.